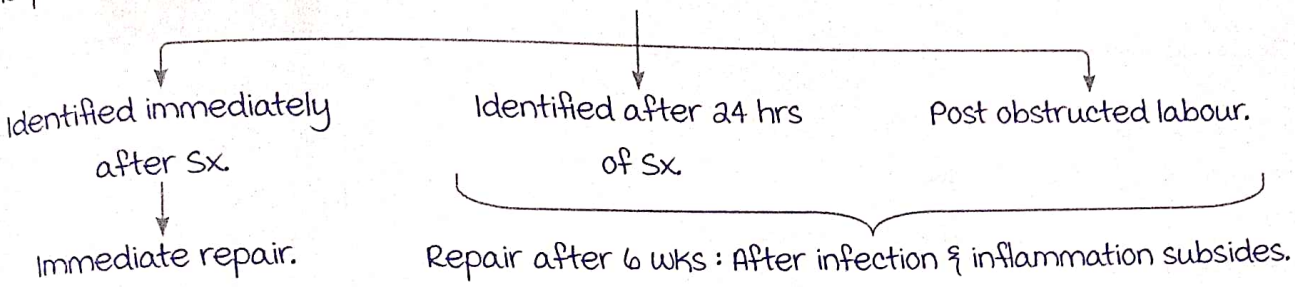
Vesicovaginal fistula :

----- Active space -----

m/c cause :

- Developed countries : Total abdominal hysterectomy (TAH).
- Developing countries : Obstructed labour.

Repair :



Technique :

- Chassar Moir technique.
- Latzko technique.

Post-op instructions :

- Bladder drainage with Foley's catheter + antibiotics x 2 wks.
- No P/S or P/V examinations. } For 3 months.
- No intercourse. } For 3 months.
- Pregnancy C/I x 1 year.

----- Active space -----

Ovarian Hyperstimulation Syndrome (OHSS)

00:30:04

Pathophysiology :

Iatrogenic complication : Inj. hMG

↓

Excessive stimulation of ovaries

↓

Multiple large follicles

↓

- Estrogen (≥2500 pg).
- VEGF from follicular fluid.

Failure of monitoring ↓

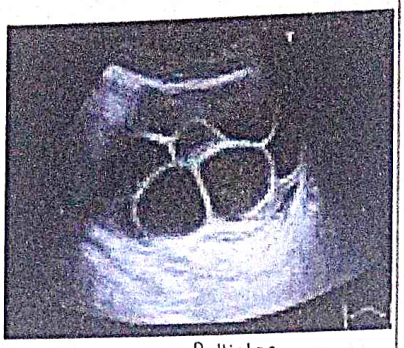
Inj. hCG

↓

Rupture of follicle

↓

Release of VEGF



Large follicles

Monitoring :

- Follicular monitoring : To monitor size of follicles.
- Estrogen levels.

Risk factors :

- Young females.
- PCOS.
- Thin female.
- ↑No. of follicles on TVS.
- ↑Antral follicle count
- ↑AMH >3.3
- ↑Estradiol (E_2) ≥2500 pg.
- Pregnancy.

OHSS

Complaints :

Nausea, vomiting, abdominal pain, dyspnoea → Clinical diagnosis.

Drugs causing OHSS :

- hMG : Highest risk.
- Clomiphene citrate : Low risk.
- Letrozole : No risk.

Triggering factors : Inj. hCG.

Mediator : VEGF.

Early OHSS :

- D/t exogenous Inj. of hCG.
- Within 9 days of Inj. hCG.

Late OHSS :

- D/t endogenous hCG produced by placenta during pregnancy.
- Inj. hCG ~~9 days~~ → ↑Endogenous production.

Prevention :

- Monitoring of follicles in TVS & estrogen levels.
- Withhold hCG if estrogen ≥2500 pg.
- Cabergoline : ↓VEGF.
- No embryo transfer : If risk of OHSS +.

Ideal conditions for hCG indication :

Estrogen : 500 – 1500 pg.

Follicles :

- Number : 2–3.
- Size : >17 mm.

Note :

Amount of E_2 released per follicle : 200 pg.